PEER REVIEW HISTORY

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ARTICLE DETAILS

TITLE (PROVISIONAL)	Screening and social prescribing in healthcare and social services to address housing issues among children and families: a systematic review
AUTHORS	Anderst, Ania; Hunter, Kate; Andersen, Melanie; Walker, Natasha; Coombes, Julieann; Raman, Shanti; Moore, Melinda; Ryan, Lola; Jersky, Michelle; Mackenzie, Amy; Stephensen, Jennifer; Williams, Carina; Timbery, Lee; Doyle, Kerrie; Lingam, Raghu; Zwi, Karen; Sheppard-Law, Suzanne; Erskine, Christine; Clapham, Kathleen; Woolfenden, Susan

VERSION 1 – REVIEW

REVIEWER	Tierney, Stephanie University of Oxford, Nuffield Department of Primary Care Health Sciences
REVIEW RETURNED	30-Jun-2021

GENERAL COMMENTS	The authors have addressed an area that is important to consider in the social prescribing landscape – housing – that may not have been given due consideration in the literature compared to other components of this approach to tackling people's non-medical needs. I feel that in the introduction, a few additional sentences are required about what is meant by social prescribing for those readers unfamiliar with the term. The end of page 4 seems to be the review's aims –but it doesn't seem to allude to social prescribing in particular – this link needs to be made. I would like to see a more clear set of aims/review question in the paper. The way the authors have phrased searches on Google (Page 7) – I wonder whether this was too specific to pick up relevant literature? Did the authors have support from an information specialist? Page 7 – did 5 authors read all abstracts? If so, this seems a bit excessive – what was the rationale for that? Page 8 and page 15 – state there is a referencing error. Figure 1 – some of the arrows don't seem to be in the correct place and some of the numbers don't add up – please check. Why was appraisal of the papers important if the authors were not looking at the effectiveness of or outcomes of studies, but tools used within them? The appraisal was not used in anyway. What was more important seemed to be the validity of tools (which was the focus of the paper) used in papers. I am not sure what the appraisal adds to the overall paper and seems to have been a redundant exercise? Is it clear who got asked questions in each paper – parents or children?

	Page 22 – can you make clear what is meant by 'Not all screening tools provide a social prescribing avenue…' Also, on page 22 – what are the ethical considerations in relation to screening and social prescribing that are alluded to? (these are mentioned later on – so perhaps just add in brackets – see below – so the reader knows that these will be discussed). Page 22, line 33 – but not all tools were specifically designed with social prescribing in mind? There is a lack of limitations section. For example, in the bullet points at the start of the paper, there is mention of the search terms being too narrow, but such limitations are not referred to in the main text.

REVIEWER	Ettinger de Cuba, Stephanie Boston University
REVIEW RETURNED	17-Nov-2021

GENERAL COMMENTS	This is a really useful systematic review of housing questions focused on children and families. It is well-written, organized and insightful. However, there are some portions that need more detail and clarification to make them more accessible for the reader and there are some odd omissions of articles that are widely cited. The search strategy outlined on page 5 of the manuscript seems clear but then doesn't seem to match what is ultimately included. The criteria include "studies that include questions about housing being asked of the population group, children or young people aged 0-18 years old and their families, accessing any health or social service. There were no restrictions on study type, design or method, and studies were not limited by country." But then later on there is discussion of that fact that only studies that were testing a screening tool were included, which doesn't match the wider criterion of "studies that include questions about housing being asked of the population group" Page 7 Lines 18-24 This section was very confusing. This needs a lot more detail to explain how you got from 8000+ articles to just over 300. And then from there to 24. Please add detail on the exclusions. After this short section, you then include a great deal of detail about the included studies, but we also need to know more about what you screened out. Especially as you screened out studies that there are referred to by other studies that you included (e.g. Cutts et al AJPH cited in Kolta et al). Other studies that surprised me with not being on the list included: https://pubmed.ncbi.nlm.nih.gov/29358482/ and https://pubmed.ncbi.nlm.nih.gov/29358482/ and the subsequent studies using this tool (most recent published just recently): https://sirenetwork.ucsf.edu/tools-resources/resources/comparing-performance-two-social-risk-screening-tools-vulnerable Also in your discussion of patient acceptability and ethics, it is strange to me that you didn't look at more updated literature as well, eg. https://www.sciencedirect.com/science/article/pii/S0749379719303186 and https://www.sciencedirect.com/science/article/pii/S0749379719303241 I would encourage looking at some more recent lit especially for the discussion as i think there has been some thinking done on these issues. e.g. https://www.jstor.org/stable/26696378 It would be good to comment on your conclusions from this review in comparison to the elements/measures suggested in this article.

	On page 14, there's a problem with one of your references in the middle of the page - the documents displays an error message. Further down on that same page, you discuss problems with utilities as a housing issue. However, I think this needs some separate discussion - as it is of course related to housing, but not housing per se and given the other dimensions of housing screening that you include, this one seems a little apart and is not always considered a housing measure. You recommend studying the validity of housing measures, but do you recommend government standardization? How might you address the contextual issues differently in Australia vs. the US? these I think would be interesting to expand on in the discussion. Thank you for this nice paper - with some minor revisions this will be an excellent contribution.

VERSION 1 – AUTHOR RESPONSE

Reviewer 1

1. "I feel that in the introduction, a few additional sentences are required about what is meant by social prescribing for those readers unfamiliar with the term.

Response: In the original submission we had described social prescribing for those who are unfamiliar with the term, as follows: “Social prescribing is a mechanism for clinicians to link or refer patients to non-medical sources of support to address social determinants of health (48, 51). Screening for housing issues has been demonstrated to increase the occurrence of social prescribing, for example referring families to appropriate professionals or providing families with resources (52, 53), and can lead to improved child health (43).”

We have added further information and an additional, recent reference from Pescheny 2020, pages 4-5.

2. "The end of page 4 seems to be the review’s aims –but it doesn’t seem to allude to social prescribing in particular – this link needs to be made. I would like to see a more clear set of aims/review question in the paper.

Response: We have made it clearer what the aims of the paper were and made the link to social prescribing within the aims, see p.5.

3. "The way the authors have phrased searches on Google (Page 7) – I wonder whether this was too specific to pick up relevant literature? Did the authors have support from an information specialist?"

Response: The search terms “social needs screening tool” and “housing screening tool for children” were chosen because these were commonly used descriptors of the types of tools we were searching for. A university research librarian at the University of New South Wales supported in the development of the search.

4. "Page 7 – did 5 authors read all abstracts? If so, this seems a bit excessive – what was the rationale for that?

Response: Thank you, we have clarified, not all 5 authors read through all abstracts, rather, all abstracts were screened twice by the reviewers, this task was shared across the six reviewers. This has been made clearer in text, see page 6.

5. "Page 8 and page 15 – state there is a referencing error."

Response: The referencing errors have been amended.

6. "Figure 1 – some of the arrows don’t seem to be in the correct place and some of the numbers don’t add up – please check.

Response: Figure 1 has been amended – there had been a calculation error. We have updated Figure 1 to include the updated search results. We would like to note that in the original manuscript, we had mistakenly reported identifying 8,323 titles excluding duplicates, when this is the number of titles including duplicates.
7. "Why was appraisal of the papers important if the authors were not looking at the effectiveness of or outcomes of studies, but tools used within them? The appraisal was not used in anyway. What was more important seemed to be the validity of tools (which was the focus of the paper) used in papers. I am not sure what the appraisal adds to the overall paper and seems to have been a redundant exercise?"
Response: The authors appraised the studies to understand the quality of the studies available in this area and guide methodological rigor for future studies.
8. "Is it clear who got asked questions in each paper – parents or children?"
Response: No, it wasn't clear whether it was parents or children who were asked the questions, however it is likely that parents were accompanying children at medical appointments and were the ones who were asked the questions about their children.
9. "Page 22 – can you make clear what is meant by 'Not all screening tools provide a social prescribing avenue…'. Page 22, line 33 – but not all tools were specifically designed with social prescribing in mind?"
Response: We have clarified this in the text. What we meant was that not all screening tools are specifically designed with the purpose to facilitate provision of information or subsequent social prescribing – some tools are simply for capturing information about patients, see page 31.
10. "Also, on page 22 – what are the ethical considerations in relation to screening and social prescribing that are alluded to? (these are mentioned later on – so perhaps just add in brackets – see below – so the reader knows that these will be discussed)."
Response: We have added the key ethical questions raised in the introduction to the discussion.
11. "There is a lack of limitations section. For example, in the bullet points at the start of the paper, there is mention of the search terms being too narrow, but such limitations are not referred to in the main text."
Response: We have added limitations to the Discussion in the main text, see p. 38.

Reviewer 2
1. "The search strategy outlined on page 5 of the manuscript seems clear but then doesn't seem to match what is ultimately included. The criteria include "studies that include questions about housing being asked of the population group, children or young people aged 0-18 years old and their families, accessing any health or social service. There were no restrictions on study type, design or method, and studies were not limited by country." But then later on there is discussion of that fact that only studies that were testing a screening tool were included, which doesn't match the wider criterion of "studies that include questions about housing being asked of the population group"
Resonse: As the authors began screening papers, it became clear that studies that include questions about housing being asked of children and/or young people were studies that involved housing questions embedded in some type of social screening tool. We have amended the wording in the Discussion to reflect that it was housing questions, and not screening tools, that were included. See p.31.
2. "Page 7 Lines 18-24 This section was very confusing. This needs a lot more detail to explain how you got from 8000+ articles to just over 300. And then from there to 24. Please add detail on the exclusions. After this short section, you then include a great deal of detail about the included studies, but we also need to know more about what you screened out. Especially as you screened out studies that there are referred to by other studies that you included (e.g. Cutts et al AJPH cited in Kolta et al)."
Response: We have included further detail regarding the process of screening and assessing eligibility, see pages 5-6.
3. "Other studies that surprised me with not being on the list included: https://pubmed.ncbi.nlm.nih.gov/29358482/

and https://pubmed.ncbi.nlm.nih.gov/29358482/ and the subsequent studies using this tool (most recent published just recently): https://sirenetwork.ucsf.edu/tools-resources/resources/comparing-performance-two-social-risk-screening-tools-vulnerable"
Response: We thank the reviewer for suggestions of studies to include in the paper. We have reviewed the studies for potential inclusion in our systematic review. We have included Sandel's 2018 Unstable Housing and Caregiver and Child Health in Renter Families as it meets inclusion criteria. Lewis' 2020 study 'Comparing the performance of two social risk screening tools in a vulnerable subpopulation' is relevant, and the screening tool Your Current Life Situation does include housing questions, however the screening tools in the study are not specifically being asked of children or families, thus do not meet our inclusion criteria. The patient/client target of our paper is children and families.
4. "Also in your discussion of patient acceptability and ethics, it is strange to me that you didn't look at more updated literature as well, eg.
https://www.sciencedirect.com/science/article/pii/S0749379719303186 and
https://www.sciencedirect.com/science/article/pii/S0749379719303241 I would encourage looking at some more recent lit especially for the discussion as i think there has been some thinking done on these issues. e.g. https://www.jstor.org/stable/26696378 It would be good to comment on your conclusions from this review in comparison to the elements/measures suggested in this article."
Response: We thank the reviewer for suggestions of relevant literature to cite in the discussion. We have cited and discussed more recent and relevant articles in the Introduction and Discussion, including the following papers:
• Cox et al. 2019 Road Map to a Unified Measure of Housing Insecurity
• Sandel et al. 2018 Unstable Housing and Caregiver and Child Health in Renter Families
• De Marchis et al. 2019 et al Part I: A Quantitative Study of Social Risk Screening Acceptability in Patients and Caregivers
• Byhoff et al. 2019 Part II: A Qualitative Study of Social Risk Screening Acceptability in Patients and Caregivers
• Husk et al. 2019 Where is the evidence?
• Chatterjee et al. 2018 Non-clinical community interventions: a systematised review of social prescribing schemes
• Chatterjee et al. 2018 Social prescribing: community-based referral in public health
• Pescheny et al. 2020 The impact of social prescribing services on service users: a systematic review of the evidence
• Berkowitz et al. 2016 Addressing basic resource needs to improve primary care quality: a community collaboration programme
5. "On page 14, there's a problem with one of your references in the middle of the page - the documents displays an error message." Response: The referencing errors have been amended.
6. "Further down on that same page, you discuss problems with utilities as a housing issue. However, I think this needs some separate discussion - as it is of course related to housing, but not housing per se and given the other dimensions of housing screening that you include, this one seems a little apart and is not always considered a housing measure."
Response: We included utilities as part of our understanding of functional and healthy housing for this paper. Access to electricity, gas and hot water are basic elements of health and wellbeing for children and families. Having functional utilities and being able to pay utility bills are material and social factors that are often included as housing measures in relation to health. We have added a clarifying sentence in the manuscript, see page 20.
7. "You recommend studying the validity of housing measures, but do you recommend government standardization? How might you address the contextual issues differently in Australia vs. the US? these I think would be interesting to expand on in the discussion."
Response: While this question is an important one, it is out of scope for this review.

VERSION 2 – REVIEW

REVIEWER	Tierney, Stephanie University of Oxford, Nuffield Department of Primary Care Health Sciences
REVIEW RETURNED	25-Feb-2022

GENERAL COMMENTS	Thank you for addressing the comments, which I feel makes for a stronger, clearer paper.

REVIEWER	Ettinger de Cuba, Stephanie Boston University
REVIEW RETURNED	23-Mar-2022

GENERAL COMMENTS	This is an extremely well-written and thoughtful paper. It will be highly useful to those in health care, social service, public health, and health services research for designing screening tools and testing questions and actions. My only comments are: 1. I was surprised that this study was not included: https://www.healthaffairs.org/doi/full/10.1377/hlthaff.2019.01569 From the description it seemed to fit your criteria. 2. The tables assessing study quality are very useful, but they don't seem to have included all the studies included in the review. It wasn't clear to me why they weren't all there. 3. Your discussion is insightful, especially regarding the ethics of asking questions without related actions. Garg et al's editorial on this topic is important and I was glad to see you cited it. The challenge is multiple – even in a situation where there are referrals to assistance, providers need to ensure that that referral is not a bridge to nowhere as housing assistance is such an underfunded resource, particularly in the US. This situation certainly burns trust with patients/clients, but also causes moral distress for providers and those who receive the referrals when they can see a problem but are not able to address it meaningfully. Your discussion made me think of this article – while it is focused on providers dealing with byzantine and punitive immigration law in health care, the impact on providers being forced to be the arbiters of cruel/nonsensical government policy is the same: https://anthrosource.onlinelibrary.wiley.com/doi/abs/10.1111/maq.12500 However, all that said, without measuring a problem, it is hard to know how big it is/what its dimensions are, so there is also an argument for screening that helps health care providers and others argue/advocate for the investment of public resources. 4. Your point regarding ensuring that screening is relevant to a given population's unique needs/context is important. It's also important to recognize which populations (not just housing issues) each question/tool successfully identifies – one size doesn't fit all on that front either (i.e. some will be better at identifying adults with housing issues vs. families – because lots of housing work/questions started with a focus on homeless adults and was NOT developed with families in mind). DeMarchis et al discuss this in their study and your point re: including people (families) with lived experience is very relevant to this.

VERSION 2 – AUTHOR RESPONSE

We thank the reviewers for the insightful comments. Regarding Reviewer 2's comments: We have now included the Bovell-Ammon paper, as it does meet our inclusion criteria, and updated the manuscript, the search flowchart and Supplementary Table 2 accordingly. We have also amended the tables assessing study quality.